DYDROGESTERONE

Widely used in pregnancy
Not contraceptive

CHEMICAL NAME 9β,10α-pregna-4,6-diene-3,20-dione (retroprogesterone)
CLASS Oral progestogen (retro-progesterone)
TRADE NAMES Duphaston · Femoston (with estradiol) · Dydroboon

THERAPEUTIC USES / INDICATIONS

- Threatened and recurrent miscarriage (luteal deficiency) — common practice in many countries.
- Luteal phase support in assisted reproduction.
- Dysmenorrhoea, endometriosis, irregular cycles, secondary amenorrhoea, dysfunctional uterine bleeding.
- Endometrial protection in HRT.

ADMINISTRATION

- **PO** with or without food, at regular intervals.

DOSAGES

- **Threatened miscarriage:** 40 mg (4 tablets) at once, then 10 mg every 8 h; continue 1 week after symptoms settle, then taper.
- **Recurrent miscarriage:** 10 mg twice daily until 20 weeks.
- **Endometriosis:** 10 mg 2–3 times daily (days 5–25 or continuous).
- **IVF luteal support:** 10 mg three times daily.

PHARMACOKINETICS

- **Peak:** 0.5–2.5 h. Bioavailability ~28%.
- Metabolised in liver to active 20α-dihydrodydrogesterone.
- Excreted in urine. **t½:** 5–7 h (metabolite 14–17 h).

PHARMACODYNAMICS

- Selective **progesterone receptor** agonist; secretory endometrium, supports pregnancy.
- No oestrogenic, androgenic, corticoid effects; does not inhibit ovulation at usual doses.

SIDE & ADVERSE EFFECTS

SIDE EFFECTS (COMMON)

- Headache, migraine.
- Nausea, breast tenderness.
- Breakthrough bleeding.

ADVERSE EFFECTS (SERIOUS)

- Liver function abnormalities (rare).
- Hypersensitivity, rash.
- Depression (rare).

CONTRAINDICATIONS & CAUTIONS

- Known or suspected progestogen-dependent tumours.
- Undiagnosed vaginal bleeding.
- Severe liver disease, liver tumours; porphyria.

KEY INTERACTIONS

- **Enzyme inducers (rifampicin, carbamazepine, St John's wort):** ↓ effect.

NURSING MANAGEMENT

- Monitor vaginal bleeding, pain; ultrasound follow-up per obstetric plan.
- Provide emotional support in pregnancy loss risk.
- Ensure correct regimen timing.

PATIENT EDUCATION

- Take at regular times; do not stop suddenly without advice in pregnancy.
- Report heavy bleeding or severe pain.
- Not a contraceptive.

KEY POINTS

- Oral retroprogesterone; no ovulation suppression.
- Endometriosis, cycle disorders.
- Threatened/recurrent miscarriage support.
- Well tolerated.

SOURCES CHECKED **[684]** Dydrogesterone SmPC (CBG-MEB, Netherlands)
Full references p. 432–457